

Screening (Substance Abuse, HIV/AIDs, Lead and Immunizations) and Referrals

Policy: Local WIC Programs should establish referral systems and provide appropriate health and social services referrals to participants.

Local WIC Programs are responsible for contacting referrals received through online referral systems or other methods within 10 days from the date of submission.

Regulation: Colorado WIC Policy, CFR 246.4 (a)(8) and (9) 246.7 (a) and (n), CFR 246.11 (a)(3) and (b)(1), CFR 246.14 (c)(1) and (9), USDA Memo 94-28

Procedures/Additional Guidance:

Referrals to WIC

The Colorado PEAK online application allows a person using PEAK to select WIC as a program they wish to be contacted by. The PEAK system creates live reports, by county, of those interested individuals' contact information. Designated local WIC agency staff manage referrals via a secure database.

The WIC Statewide Referral Form is an electronic form community partners and the public can use to submit requests for WIC to contact potentially-eligible WIC participants. Designated local WIC agency staff manage referrals via a secure database.

Development of Referral Systems

The establishment of a local agency referral network consists of two components: identification of community resources and education of WIC staff on community resources.

- A local referral network includes programs housed within the same agency as the WIC Program (intra-agency), as well as other community organizations (inter agency). The number of community agencies within the WIC Program service area will determine the size of the referral network.
- To ensure participants are appropriately directed to other community services, it is necessary to inform all WIC staff about the organizations within the referral network. WIC staff should understand the purpose/function and eligibility criteria for each agency/organization in the network.
 - Several mechanisms could be used to inform the WIC staff members. Agency representatives can be invited to provide a short presentation at WIC staff meetings. Pamphlets from the various community organizations could be made available to the WIC staff. Maintaining a "community resource" notebook with these pamphlets would provide the staff with easy access.

Referrals can be informal (informing the client about the SNAP Program) or formal (sending a standardized referral form). This will depend on the reason for the referral. If the referral includes client information or requires local staff to contact the local program or organization on behalf of the family, the participant or endorser's signature of consent must be obtained. If the endorser has granted permission to release information to the physician based on their answer provided in the Nutrition Interview, client information may be released to the physician. Referrals may be printed and given to the participant upon request.

Making Referrals

Agencies have the option to provide either an informal or formal referral. Methods for each type of referral are described as follows:

- Informal Referral: A referral can be accomplished simply by providing all adult participants and caretakers of participating infants and children with the list of available resources.
- Formal Referral: A local agency may opt to develop a more formal referral system wherein a staff member initiates contact with a resource (e.g. substance abuse counseling or treatment agency) on behalf of an interested participant. Please note: Confidentiality rules must be followed.

It is recommended that all clients be directed informally to:

- The Supplemental Nutrition Assistance Program (SNAP)

- Medicaid (Health First Colorado)
- Child Health Plan Plus (CHP+)
- Temporary Assistance to Needy Families (TANF) Programs,
- Colorado PEAK at <https://coloradopeak.secure.force.com> (an on-line service for Coloradans to check for potential food and cash assistance programs, apply for Medicaid and Child Health Plan Plus programs, WIC, SNAP, TANF and Aging and Adult Services Financial programs and manage benefits on-line)

Staff should provide the addresses, web address and/or telephone numbers for these referral resources. The local WIC Program can obtain this information from the Social Services Agency.

Documenting Referrals

All referrals by local WIC staff persons must be entered on the *Referrals - Participant* panel in the Compass system or noted in the participant's Care Plan. All written information given to participants on assistance programs must be recorded.

Referral Follow-Up

To ensure that quality nutritional care is provided to WIC clients, follow up should be provided on all referrals.

Substance Abuse Information and Referral

The Colorado WIC Program screens for drug, alcohol, and tobacco use to determine nutrition/medical criterion through the use of questions asked on the Nutrition Interview. WIC staff must provide information on the dangers of drugs and other harmful substances and referrals to pregnant, postpartum, and breastfeeding women and to the parents and caretakers of WIC infants and children. Participants who indicate that they or their child are exposed to secondhand tobacco smoke must also be warned of the dangers to themselves and/or their children. Information must be provided in verbal form. Visuals, written communication and online resources may supplement the information provided verbally. Staff members are not expected to diagnose drug and other substance abuse or to provide in-depth counseling. Rather, they are to provide information on the dangers of substance abuse and secondhand smoke in the course of routine activities.

Federal Drug and Other Harmful Substance Abuse Information and Referral regulations specify the following requirements for all local WIC Programs:

- Participants with suspected drug or other harmful substance abuse problems must be referred to an appropriate counseling, treatment or education program where services are locally available.
- Each local agency must maintain and make available for distribution a list of local resources for substance abuse counseling and treatment.
 - (e.g., Alcoholics Anonymous, Narcotics Anonymous, Cocaine Anonymous, Al-Anon Family Groups, Adult Children of Alcoholics, Women for Sobriety), drug and alcohol treatment programs as well as local smoking cessation programs sponsored by organizations such as the American Lung Association and American Cancer Society. In communities where resources are limited, the public health nurse or the participant's physician should be considered as a possible resource.
- Local agencies are required to coordinate with local alcohol and drug abuse treatment services.

HIV/AIDS Information and Referral

The Colorado WIC Program can play an important role in preventing the transmission of HIV/AIDS in the perinatal population. The following guidelines define the policy for the WIC Program regarding issues related to HIV/AIDS and WIC participants:

- Encourage comprehensive health care for all WIC participants.
- Advise all pregnant, postpartum, and breastfeeding women to know their HIV status and receive early diagnosis through routine, voluntary testing, and counseling.
- Provide appropriate referrals to participants for testing, counseling, and medical treatments as needed.
- Strongly encourage and support breastfeeding as the healthiest way to feed infants.
- Inform all participants that HIV-positive mothers should not breastfeed, and that HIV is more easily transmitted through breast milk during the period immediately following the onset of infection.
 - The Colorado WIC Program concurs with the Colorado Department of Public Health and Environment and United States Department of Agriculture to support the recommendations of the American

Academy of Pediatrics and the Centers for Disease Control and Prevention that women in the United States should not breastfeed if they are HIV positive or have AIDS.

Blood Lead Screening Information & Referral

To implement the provision in Public Law 106-387, included in the WIC Appropriations Act FY2001, local WIC Programs are required to:

- Ask if the child has had a blood lead screening test starting at 12 months of age.
- If the child has not had a test, they must be referred (at each certification/midcert/recertification) to programs where they can obtain such a test.
- Document lead screening referrals in the Nutrition Education Panel by selecting "Referred for Lead Screening" under child standard counseling.
- Once a lead test has been performed and documented, screening and referral is no longer required.

The blood screen within the Assessment panel is where staff record if a child has received a blood lead screening test. In Colorado, lead testing is covered by Medicaid and the Child Health Plan Plus (CHP+). Additionally, most private insurance providers will cover the cost of the testing when the test is ordered by a physician. Some local health departments may also provide lead testing. Public Law 106-387 does not allow WIC funds to be used to conduct blood lead screening tests. A hematological test for anemia, such as a hemoglobin or hematocrit, is the only laboratory test required to determine a person's eligibility for WIC. It is therefore, the only cost that may be charged to WIC.

Clinical Laboratory Improvement Amendments (CLIA) Certification Number: Clinical laboratories are registered under the Clinical Laboratory Improvement Amendments (CLIA) federal regulation to ensure quality laboratory testing. All labs that perform testing on human specimens for diagnosis, treatment or health assessment in the United States must hold the appropriate CLIA certificate. The CLIA regulations set standards for personnel, records, quality control and quality assurance.

- Each WIC local agency must operate under a CLIA Certificate. This CLIA Certificate is typically held by the Health Department, not by the WIC Program. Each WIC clinic needs to be listed on the CLIA Certificate, however if a local agency does not know their CLIA Certificate number (typically starts with "06D"), the WIC director may ask the Public Health director, lab manager or nurse manager in their agency. If an agency is unsure of their CLIA number, contact:

Jeff Groff
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EBAT and Laboratory Certification
P 303.692.3681 | F 303.344.9965
8100 Lowry Blvd, Denver, CO 80230
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Immunization Screening, Information, and Referral

Low-income children are less likely to be immunized than their higher-income counterparts, placing them at high risk for potentially serious diseases, such as diphtheria, pertussis, poliomyelitis, measles, mumps, and rubella. WIC staff members play a significant role in helping to improve the immunization rates for low-income infants and children by assessing immunization records, educating participants on the importance of immunizations and by making referrals as necessary to bring immunizations up-to-date.

Local WIC agencies are required to:

- Ask caregivers of infants and children under 25 months of age to bring immunization records to all certification/recertification and mid-certification visits. Note: If the record is available in the Colorado Immunization Information System (CIIS), agencies with access to CIIS may access electronic records in lieu of obtaining the immunization information from the caregiver.
- Assess immunization records for infants and children less than 25 months of age for recommended DTaP shots.
 - ✓ 1st DTaP is at 2 months of age
 - ✓ 2nd DTaP is at 4 months of age
 - ✓ 3rd DTaP is at 6 months of age
 - ✓ 4th DTaP is between 15 and 18 months of age

- Provide information on the immunization schedule and refer caregivers to obtain immunizations if the infant or child's immunizations are not up-to-date or the immunization record is not available.
- Once a child has received their 4 DTaP immunizations, staff no longer need to ask about the child's immunization status.

Referral Resources

Referral Resource	General Purpose/Function
<i>Alcohol & Drug Abuse</i>	Prevention counseling and comprehensive treatment for substance abuse
<i>Childbirth Education Associations</i>	Prenatal education
<i>Child Health Plan Plus (CHP+)</i>	Medical Assistance for low income children who are not eligible for Medicaid
<i>Colorado State University Extension Service</i>	Consumer classes, food preparation information, recipes
<i>Early & Periodic Screening, Diagnosis & Treatment (EPSDT) Program</i>	Nutrition education
<i>Family Planning/Planned Parenthood</i>	Guidance and counseling to provide freedom of choice in the number and spacing of children
<i>Supplemental Nutrition Assistance Program</i>	Food assistance for low income
<i>Genetic Counseling/Screening</i>	Genetic counseling, diagnosis and education primarily for PKU, hypothyroidism, galactosemia, sickle cell anemia, homocystinuria, maple syrup urine disease
<i>Health Care Program for Children with Special Needs (HCP)</i>	Diagnosis and treatment services for special needs children
<i>Head Start</i>	Preschool educations for disadvantaged children
<i>Housing Authority</i>	Housing information/placement for low income
<i>Immunization Clinics</i>	Immunizations
<i>La Leche League</i>	Breastfeeding support
<i>Local Physicians, Medical Clinics, Hospitals</i>	Medical care
<i>Maternity Services</i>	Prenatal, postnatal comprehensive care
<i>Medicaid</i>	Medical assistance for low income women and children
<i>Mental Health Services</i>	Prevention and crisis intervention counseling
<i>Prenatal Plus</i>	Assistance for high-risk women who are on Medicaid
<i>Temporary Assistance to Needy Families</i>	Financial assistance for low income children



Referral Resources based on Condition/Problem

Condition/Problem	Referral Source
<i>New Migrant Family</i>	Farm Worker Health Services Program
<i>Infant/Child w/Cleft Palate or other Handicapping Conditions</i>	Health Care Program for Children with Special Needs (HCP)
<i>Infant/Child w/Failure to Maintain Growth Percentile</i>	Public Health Nurse
<i>An individual or family needing food assistance</i>	Supplemental Nutrition Assistance Program
<i>Inadequate Food Supply</i>	Local Food Banks
<i>Breastfeeding Women</i>	La Leche League
<i>Overweight Postpartum Women</i>	Weight control classes
<i>Women entering with Short Inter-Conceptual Period of High Parity</i>	Family Planning/Planned Parenthood
<i>Pregnant Women for HIV Testing</i>	Planned Parenthood, Private Physician, Local Health Department
<i>Pregnant Women who have not received Prenatal Care</i>	Local Prenatal Program, local private physician accepting Medicaid
<i>Pregnant Women who Smoke</i>	A community smoking cessation program Colorado Quitline 1.800.QUIT.NOW (1.800.784.8669) https://www.coquitline.org
<i>A Severely Anemic Participant</i>	Public Health Nurse, local physician
<i>Teenage Mother or Teenage Pregnant Women</i>	Public Health Nurse, Community prenatal support groups